

# Sante Speech Health Summary

Report ID: sum\_fb309b723e9e | Generated: 2026-02-15 15:48 UTC

Session: conversation | Duration: 124.9s

## Executive Summary

Data quality is high; exploratory interpretations are likely stable.

- Intoxicated Speech Pattern Signal: low (score 12)
- Sick / Tired Voice Pattern Signal: low (score 9)
- Aphasia-like Language Pattern Flag: low (score 8)

## Estimates

### Intoxicated Speech Pattern Signal | low | score 12

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Short-pause index 0.00; Slow-rate index 1.00; Loudness mean 0.369; Jitter 0.023

Limitations: Sleep deprivation, stress, and microphone clipping can mimic this pattern.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

### Sick / Tired Voice Pattern Signal | low | score 9

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Long-pause index 0.04; Energy-drop index 0.53; Low-loudness index 0.41; HNR 5.18 dB

Limitations: Cannot separate respiratory causes from fatigue without clinical context.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

### Stress Activation Speech Signal | low | score 7

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Speech-rate proxy 1.85 peaks/s; Short-pause index 0.00; Pressured-tempo index 0.25; Fast-rate index 0.00

Limitations: High arousal can also reflect urgency, excitement, or speaking style.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

### Aphasia-like Language Pattern Flag | low | score 8

Suggestion: Low signal in this sample. Monitor trend across repeated sessions and retest with a matched microphone/device/codec setup before drawing conclusions.

Evidence: Phoneme coverage 732 tokens; Flagged disfluencies 161; Voiced segment rate 2.03/s; Lexical diversity 0.67

Limitations: Cannot subtype aphasia from this short speech sample.; Exploratory speech biomarkers are probabilistic, not diagnostic outcomes.

## Transcript Snapshot

Okay, is it, you're listening to me right now? Okay, I am having a fantastic time here at Tree Hacks. I'm enjoying checking out the demos of the hardworking hackers and seeing what they built. I don't know how much longer I need to talk, but I'm gonna keep talking. It would be nice if there were some sort of reading thing that I could read, but I don't know that probably modulates your voice a little bit if you're reading out loud and it's naturally talking, or maybe some sort of prompt, like, tell me about your dad. What's your dad like, or something like that. Okay, well, I've been talking for about a minute or so. Open your The coolest demo has been this one, because I'm very impressed with the notion of being able to evaluate one's health just through listening to one's voice. I don't have a smoker cough. I don't have like a grizzled, kind of like... You know, case of whiskey a month type voice, do I? I would love to find some more tea, but I can

only find coffee, and coffee just has slightly too much kick for me at this type of event. Mhm. Please open the window a bit. My favorite color changes every season.

*Research-only estimate report. It is not a medical diagnosis. Use in combination with formal clinical assessment and environmental quality checks.*